

FRONT

hovid

hovid-Lambivir Tablet 150 mg / 300 mg

VILAMxx-PC1

DESCRIPTION

Caplet-shaped white to off-white film-coated tablet, bevel-edged, shallow convex with break bar embossed on one face.

COMPOSITION

Each film-coated tablet contains 150 mg of lamivudine and 300 mg of zidovudine.

PHARMACODYNAMICS

Lamivudine and zidovudine are nucleoside analogues which have activity against HIV. Additionally, lamivudine has activity against hepatitis B virus (HBV). Both medicinal products are metabolised intracellularly to their active moieties, lamivudine 5'-triphosphate (TP) and zidovudine 5'-TP respectively. Their main modes of action are as chain terminators of viral reverse transcription. Lamivudine-TP and zidovudine-TP have selective inhibitory activity against HIV-1 and HIV-2 replication in vitro; lamivudine is also active against zidovudine-resistant clinical isolates of HIV. Lamivudine in combination with zidovudine exhibits synergistic anti-HIV activity against clinical isolates in cell culture.

PHARMACOKINETICS

- **Lamivudine:** Following oral administration, lamivudine is rapidly absorbed and extensively distributed. Binding to plasma protein is low. Approximately 70% of an intravenous dose of lamivudine is recovered as unchanged drug in the urine. Metabolism of lamivudine is a minor route of elimination. In humans, the only known metabolite is the trans-sulfoxide metabolite (approximately 5% of an oral dose after 12 hours).
- **Zidovudine:** Following oral administration, zidovudine is rapidly absorbed and extensively distributed. Binding to plasma protein is low. Zidovudine is eliminated primarily by hepatic metabolism. The major metabolite of zidovudine is GZDV. GZDV area under the curve (AUC) is about 3-fold greater than the zidovudine AUC. Urinary recovery of zidovudine and GZDV accounts for 14% and 74% of the dose following oral administration, respectively. A second metabolite, 3'-amino-3'-deoxythymidine (AMT), has been identified in plasma. The AMT AUC was one-fifth of the zidovudine AUC.

INDICATION

Lambivir is indicated in antiretroviral combination therapy for the treatment of Human Immunodeficiency Virus (HIV) infection.

CONTRAINDICATIONS

Lambivir Tablets are contraindicated in patients with previously demonstrated clinically significant hypersensitivity (e.g., anaphylaxis, Stevens-Johnson syndrome) to any of the components of the product.

WARNINGS AND PRECAUTIONS

- Hematologic toxicity/bone marrow suppression including neutropenia and anemia have been associated with the use of zidovudine, one of the components of Lambivir.
- Symptomatic myopathy associated with prolonged use of zidovudine.
- Lactic acidosis and hepatomegaly with steatosis, including fatal cases, have been reported with the use of nucleoside analogues including zidovudine. Suspend treatment if clinical or laboratory findings suggestive of lactic acidosis or pronounced hepatotoxicity occur.
- Acute exacerbations of hepatitis B have been reported in patients who are co-infected with hepatitis B virus (HBV) and human immunodeficiency virus (HIV-1) and have discontinued lamivudine, a component of Lambivir. Monitor hepatic function closely in these patients and, if appropriate, initiate anti-hepatitis B treatment.
- Lambivir should not be administered with other lamivudine- or zidovudine-containing products or emtricitabine-containing products.
- Hepatic decompensation, some fatal, has occurred in HIV-1/HCV co-infected patients receiving combination antiretroviral therapy and interferon alfa with/ without ribavirin. Discontinue Lambivir as medically appropriate and consider dose reduction or discontinuation of interferon alfa, ribavirin, or both.
- Exacerbation of anemia has been reported in HIV-1/HCV co-infected patients receiving ribavirin and zidovudine. Co-administration of ribavirin and zidovudine is not advised.
- Pancreatitis: Use with caution in pediatric patients with a history of pancreatitis or other significant risk factors for pancreatitis. Discontinue treatment as clinically appropriate.
- Immune reconstitution syndrome and redistribution / accumulation of body fat have been reported in patients treated with combination antiretroviral therapy.

PREGNANCY AND LACTATION

Lambivir should be used during pregnancy only if the potential benefit justifies the potential risk to the fetus. Mothers should be instructed not to breastfeed if they are receiving Lambivir.

ADVERSE EFFECTS / UNDESIRABLE EFFECTS

Adverse reactions have been reported during therapy for HIV disease with lamivudine and zidovudine separately or in combination. For many of these events, it is unclear whether they are related to lamivudine, zidovudine, the wide range of medicinal products used in the management of HIV disease, or as a result of the underlying disease process.

As Lambivir contains lamivudine and zidovudine, the type and severity of adverse reactions associated with each of the compounds may be expected. There is no evidence of added toxicity following concurrent administration of the two compounds.

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BACK

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General

Redistribution/accumulation of body fat, vasculitis, weakness, malaise, fatigue, fever

Cardiovascular

Cardiomyopathy

Endocrine and Metabolic

Gynecomastia, hyperglycemia

Gastrointestinal

Nausea, vomiting, abdominal pain or cramps, diarrhea, oral mucosal pigmentation, stomatitis

Blood and lymphatic system

Anemia (including pure red cell aplasia and anemias progressing on therapy), lymphadenopathy, splenomegaly, neutropenia, thrombocytopenia, leucopenia

Hepatic and pancreatic

Lactic acidosis, hepatic steatosis, pancreatitis, post-treatment exacerbation of Hepatitis B

Hypersensitivity

Sensitization reactions (including anaphylaxis), urticarial

Musculoskeletal

Muscle weakness, rhabdomyolysis, arthralgia, myalgia

Nervous system

Paresthesia, peripheral neuropathy, seizures, headache, insomnia

Respiratory

Abnormal breathing sounds/ wheezing, cough, nasal symptoms

Skin

Alopecia, rash, erythema multiforme, Steven-Johnson syndrome

DRUG INTERACTIONS

- **Concomitant use with the following drugs should be avoided:**
stavudine, zalcitabine and doxorubicin.
- **Bone marrow suppressive/ cytotoxic agents:**
May increase the hematologic toxicity of zidovudine

OVERDOSE AND TREATMENT

There is limited experience of overdosage with Lambivir. No specific symptoms or signs have been identified following acute overdose with zidovudine or lamivudine apart from those listed as undesirable effects. No fatalities occurred, and all patients recovered.

If overdosage occurs the patient should be monitored for evidence of toxicity and standard supportive treatment applied as necessary. Since lamivudine is dialysable, continuous haemodialysis could be used in the treatment of overdosage, although this has not been studied. Haemodialysis and peritoneal dialysis appear to have a limited effect on elimination of zidovudine, but enhance the elimination of the glucuronide metabolite. For more details, physicians should refer to the individual prescribing information for lamivudine and zidovudine.

DOSAGE AND ADMINISTRATION

Lambivir may be administered with or without food.

Adults and Adolescents Weighing ≥ 30 kg:

1 tablet twice daily.

Pediatric Patients:

Dosage should be based on body weight not to exceed adult dose.

Patients Requiring Dosage Adjustment:

Because Lambivir is a fixed-dose combination tablet, it should not be prescribed for pediatric patients weighing less than 30kg or patients requiring dosage adjustment, such as those with reduced renal function (creatinine clearance less than 50 mL/min), patients with hepatic impairment, or patients experiencing dose-limiting adverse reactions.

Note: The information given here is limited. For further information consult your doctor or pharmacist.

Storage: Store below 30°C.

Presentation/ Packing: Blister pack of 6 x 10's, 1 x 10's.

Product Registration Holder: HOVID Bhd.

121, Jalan Tunku Abdul Rahman, 30010 Ipoh, Malaysia.

Manufactured by: HOVID Bhd.

Lot 56442, 7¹/₂ Miles, Jalan Ipoh/Chemor, 31200 Chemor, Malaysia.

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